

REFORM DEMOCRATS OF THE NEW COMMONWEALTH

Scoring Methodology · Framework v26

PROPOSAL SCORING METHODOLOGY

Definitions, Evidence Standards, and Rubrics for the Three-Axis Framework

REF_RDNC_ScoringMethodology_2026
v1.1 · May 2026 · Internal Reference — REF_Layer

Document ID	REF_RDNC_ScoringMethodology_2026 · v1.1
Classification	REF_Layer · Internal + Trusted Partners
Framework version	Applies to Framework v26 (65 proposals; avg score 7.4/10.0)
Scoring axes	Global Precedent (GP) · US Transition Difficulty (UT) · Benefit Score (B) · Each axis: 1–5 · Aggregate: 1–10
Companion documents	REF_RDNC_Framework_v26_Sourced_2026_5 · REF_RDNC_SourceReferenceGuide_2026_11 · REF_RDNC_ConstitutionalResearch_2026
Status	Canonical — adopted methodology of record for Framework v26

CANONICAL METHODOLOGY

This document is the adopted methodology of record for Framework v26. Scores in Framework v26 are derived using the rubric definitions, evidence tiers, and aggregation formula described herein. Prior versions of the framework (v25 and earlier) used an informal methodology that this document formally replaced. The rubric remains subject to periodic revision as the evidence base evolves; revision history is logged with each iteration. This is not a legal opinion and does not constitute professional consulting advice.

Section 1 — Purpose and Design Philosophy

Why a Formal Scoring Methodology Matters

The Reform Democrats of the New Commonwealth operates as a policy research institution — a think tank, in functional terms — producing legislative frameworks, comparative analyses, and a governing agenda designed to be adopted by a future presidential administration. That institutional role carries a corresponding methodological obligation: the scoring of proposals in the ARA Framework must be defensible to the same standards applied to comparable work by the Brookings Institution, the Urban Institute, the Congressional Budget Office, or the Heritage Foundation.

The three-axis scoring framework (GP · UT · B) makes an implicit claim about each of the 65 proposals: that the score reflects an evidence-based assessment of how well the policy has worked internationally, how achievable and impactful it is in the US context, and how significant its welfare benefits are. Without explicit rubric definitions, those claims rest on unstated assumptions. This document makes those assumptions explicit.

Design Principles

The rubric is designed to satisfy five principles:

- **Operational precision:** each score tier has explicit, observable criteria that distinguish it from adjacent tiers without ambiguity.
- **Evidence grounding:** each score tier implies a minimum evidence standard drawn from established research quality frameworks (GRADE methodology, CBO evidence tiers, OECD evaluation standards).
- **Intellectual honesty:** the rubric acknowledges the limits of cross-national comparison, the uncertainty in forward-looking projections, and the value-laden nature of welfare assessment. Limitations are stated, not obscured.
- **Calibrated conservatism:** where evidence is ambiguous, the rubric defaults to the lower score. Optimism must be substantiated; pessimism does not require justification.
- **Reproducibility:** a trained policy analyst applying this rubric to the same proposal and source set should arrive at the same score (± 0.5) as any other such analyst.

Score Aggregation Formula

Each of the three axes is scored 1 to 5. The aggregate score, reported on a 1-10 scale, is computed as:

$$\text{Aggregate Score} = (\text{GP} + \text{UT} + \text{B}) \div 1.5$$

Minimum: $3 \div 1.5 = 2.0$ · **Maximum:** $15 \div 1.5 = 10.0$ · **Step:** 0.5 points per half-point on any axis

Scores are reported to one decimal place. Where a proposal falls at a boundary between two tiers (e.g., GP borderline between 3 and 4), the lower score is assigned unless the evidence clearly resolves the ambiguity in favor of the higher score. Score rationale must be documented in the per-proposal justification record for any score above 3 on any axis.

Relationship Between the Three Axes

The three axes are fully independent and genuinely orthogonal — each measures something the others do not. GP measures what happened elsewhere; UT measures how hard it is to make it happen here; B measures what it is worth when it does. The pairwise comparisons are each analytically useful: GP and UT together make the case that something has worked internationally and can work in the United States. GP and B together make the case that something has worked internationally and delivers real welfare improvement. UT and B together reveal the central policy tension: how difficult is implementation versus how valuable is the outcome. It was a deliberate design decision that UT not measure benefit magnitude — that is the role of B. Conflating implementation difficulty with benefit magnitude would collapse two orthogonal dimensions into one, reducing the analytical power of the framework.

Important: Independence of Axes

GP and UT in particular can diverge significantly and appropriately. A proposal might have GP=1 (no international precedent) and UT=4 (strong US-specific case and implementation pathway) if the evidence base is primarily from domestic modeling rather than international experience. Such a divergence should be noted explicitly in the scoring rationale rather than resolved by artificial averaging.

Section 2 — Comparator Nations and Evidence Quality Tiers

The Comparator Nation Pool

GP scoring requires assessing whether a policy has "worked in comparable nations." The following definition operationalizes "comparable":

Tier	Qualifying Criteria	Primary Nations Included
Tier 1 — Primary Comparators	GDP/capita ≥ \$30,000 PPP · V-Dem Liberal Democracy Index ≥ 0.5 · OECD member · Functional welfare state and rule of law	Australia, Austria, Belgium, Canada, Denmark, Finland, France, Germany, Ireland, Japan, Netherlands, New Zealand, Norway, Sweden, Switzerland, United Kingdom (16 nations)
Tier 2 — Secondary Comparators	High-income democracies with differing institutional contexts; evidence valid but transferability requires explicit justification	Czech Republic, Estonia, Iceland, Israel, Luxembourg, Portugal, Singapore, Slovakia, Slovenia, South Korea, Spain (digital, energy, and specialized policy areas)
Non-Comparable	Authoritarian states, low-income nations, nations with institutional capacity fundamentally incompatible with US context. Evidence from these contexts does not qualify for GP scoring and should not be cited as primary precedent.	China, Russia, Gulf states, most of Africa, Latin America, and Asia (except as Tier 2 where specified). May appear as supporting evidence but cannot anchor a GP score.

Scoring note: A policy's nation count is assessed at time of meaningful implementation and sustained operation (minimum 3 years), not at date of passage. A law on the books that has not been implemented does not count toward the comparator tally.

Evidence Quality Tiers

All three axes require evidence to justify scores above 2. The following tiers define what qualifies as supporting evidence at each level. These tiers are adapted from the GRADE (Grading of Recommendations, Assessment, Development and Evaluations) framework and CBO methodology documentation.

Tier	Label	Description and Qualifying Source Types
A	High Quality	Peer-reviewed longitudinal studies with comparison groups (≥5 years); randomized policy evaluations; government program evaluations using difference-in-differences or IV methods; OECD/WHO systematic reviews; CBO or equivalent statutory scoring with explicit model documentation. Evidence direction is consistent across multiple independent studies.
B	Moderate Quality	Well-documented program reports with clear methodology; cross-sectional comparative analyses with appropriate controls; peer-reviewed studies with methodological limitations (short follow-up, single country, self-selection concerns); established think tank evaluations (Brookings, Urban Institute, RAND) with transparent assumptions. Some evidence may conflict.
C	Low Quality	Case studies without control groups; advocacy organization reports; government press releases; news coverage; preliminary implementation data (<2 years); modeling studies with opaque assumptions; extrapolations from tangentially related programs. Useful for context; insufficient to anchor scores above 2.

D	Non-Qualifying	Opinion pieces, partisan advocacy, anecdote, unreviewed working papers, sources with clear conflicts of interest, self-reported outcomes without independent verification. May be cited for context but carry zero evidentiary weight for scoring purposes.
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Section 3 — Axis 1: Global Precedent (GP)

Has this policy approach been adopted and rigorously evaluated in comparable nations?

The GP axis measures whether the policy has an international evidence base. It asks: given what happened in comparable nations when they tried this approach, what should we expect? GP is backward-looking — it assesses what has already occurred, not what might occur in the US.

A high GP score does not guarantee the policy will succeed in the US. Institutional differences, constitutional constraints, and political economy vary. GP tells us whether we are walking into territory that others have successfully mapped, or whether we are building from theory.

Score	Tier Label	Qualifying Criteria (must satisfy primary condition)	Minimum Evidence Standard	US Adaptation Barrier
5	Overwhelming Precedent	≥75% of Tier 1 comparators have adopted the policy at national scale AND sustained it for ≥10 years; OR the approach constitutes an international norm endorsed by OECD, WHO, ILO, or equivalent standards body. Multiple Tier A independent longitudinal evaluations confirm consistent positive outcomes across diverse national contexts. Failed implementations are rare (fewer than 2 of 16 Tier 1 nations) and are well-understood exceptions.	Tier A evidence (multiple independent studies). No significant contested findings in peer-reviewed literature.	None or minor. Implementation pathway in comparable federal systems well-documented.
4	Strong Precedent	5+ Tier 1 comparators have adopted and sustained the policy; OR 2+ Tier 1 nations with Tier A longitudinal evaluation showing consistent positive outcomes. Overall evidence direction is clearly positive. Manageable implementation challenges are well-understood.	At minimum: 2 Tier B evaluations or 1 Tier A evaluation. Evidence direction consistently positive.	Clear pathway exists. US-specific adaptation challenges are identified and not prohibitive.
3	Moderate Precedent	3–4 Tier 1 comparators have implemented the policy with generally positive but contested results; OR 2+ Tier 1 nations with Tier B evaluation evidence. Evidence shows promise but contains significant caveats: effects concentrated in specific demographics, fiscal sustainability questions, or mixed outcomes across implementation sites.	At minimum: 1 Tier B evaluation. Peer-reviewed literature acknowledges the approach as viable.	Meaningful adaptation required. Some structural concerns exist but are not disqualifying.
2	Emerging Precedent	1–2 Tier 1 comparators with mixed or early results; OR 3+ Tier 2 comparators without clear applicability to US context; OR comparable nations attempted the policy but at a fundamentally different scale. Results are inconclusive or contested in the peer-reviewed literature.	At minimum: 1 Tier C evaluation or 2 Tier D sources corroborating the general direction. Low-quality evidence is ceiling.	Significant adaptation barriers. Evidence of transferability is limited.

Score	Tier Label	Qualifying Criteria (must satisfy primary condition)	Minimum Evidence Standard	US Adaptation Barrier
1	No Meaningful Precedent	Policy has not been implemented in any Tier 1 or Tier 2 comparator at scale; OR only precedent is in non-comparable contexts (authoritarian states, low-income nations); OR policy has been attempted and clearly failed in the majority of cases. Includes genuinely novel policies where the US would be the first mover.	Tier C or D only; or no external evidence at all. Score 1 is not a criticism — it accurately describes the evidentiary state.	Substantial or unknown. Policy is entering unmapped territory.

GP Scoring Note: Scale Matters

A policy may be implemented in many nations but at a fundamentally different scale than what is proposed for the US. The Medicare program covers ~65M Americans; comparable universal healthcare systems cover entire national populations. When scale differs dramatically, the GP score should reflect what evidence exists at the relevant scale, not merely at any scale. Include a note in the scoring rationale when this consideration is material.

Section 4 — Axis 2: US Transition Difficulty (UT)

How difficult is the transition from the current US status quo to this policy being operational at meaningful scale?

The UT axis asks a specific, US-focused question: how difficult is it to move from the current status quo to this policy being enacted, implemented, and operational at meaningful scale in the United States? It is assessed across four dimensions: (a) constitutional and legal feasibility; (b) legislative pathway and political barriers; (c) administrative capacity, timeline, and US precedent; and (d) agenda synergy with other ARA proposals. A higher score means the transition is more achievable; a lower score means it faces fundamental barriers. UT is not a measure of how beneficial the policy would be if implemented — that is captured by B. UT measures the gap between here and operational.

UT is deliberately US-specific and process-focused. A policy with strong international precedent (high GP) may still face extreme US transition difficulty (low UT) due to constitutional constraints, political economy, or administrative capacity gaps. Conversely, a policy with no international precedent can have high UT if the domestic implementation pathway is clear. UT and B are fully independent: a policy that is easy to implement (high UT) may deliver modest benefit (low B); a policy that would deliver transformative benefit (high B) may face near-impossible transition barriers (low UT=1). It is the interplay of all three axes that produces the full picture.

Why UT Was Previously Under-Sourced and Why the Axis Definition Matters

Prior versions of the framework derived UT scores primarily from the Sequencing Map — an internal operational document. This created a circular reference: the framework scored UT using an RDNC document that was itself derived from the framework. This methodology document replaces that approach. UT scores must now be substantiated by independent evidence: CBO/PWBM/Moody's economic modeling, legislative feasibility analysis, implementation timeline studies, GAO implementation assessments, or prior US program experience. The key question for sourcing is always: what does the evidence tell us about whether and how quickly the US can make this transition — not about what the benefit would be if it did.

UT is assessed along four sub-dimensions, each of which can independently prevent a high UT score:

- **Legislative and constitutional feasibility:** the realistic pathway to enactment given the US legislative process, constitutional constraints, and political economy.
- **Implementation timeline:** how quickly, after enactment, can the policy be made operational at meaningful scale — not whether the outcomes are positive, but whether the mechanics of implementation are achievable within a defined horizon.
- **US precedent and scaling:** what does prior US domestic experience (from analogous programs, phased rollouts, or administrative expansions) tell us about whether this type of transition is achievable and at what speed?
- **Agenda synergy:** does this proposal create enabling conditions for other ARA proposals, or does it operate in isolation? Synergistic proposals that unlock other proposals score higher.

Score	Tier Label	Qualifying Criteria (must satisfy primary condition)	Minimum Evidence Standard	US Adaptation Barrier
5	Minimal Transition Difficulty	Implementation achievable through existing statutory authority or simple majority legislation. Precedent in US domestic programs is strong and directly applicable. Timeline to operational: 1-2 years from enactment. No significant constitutional, legal, or administrative barriers exist. Strong agenda synergy — this proposal enables or significantly amplifies 5+ other ARA proposals.	Convergent Tier A/B evidence from ≥2 independent sources (CBO, PWBM, academic modeling, or domestic program scaling data) supporting transformative positive trajectory. Fiscal multiplier ≥1.5 where applicable.	No constitutional barrier. Administrative capacity exists or can be built within implementation window.

Score	Tier Label	Qualifying Criteria (must satisfy primary condition)	Minimum Evidence Standard	US Adaptation Barrier
4	Low Transition Difficulty	Implementation achievable through reconciliation, simple majority legislation, or executive action with existing administrative authority. US domestic program precedent is relevant and supportive. Timeline to operational: 2–5 years from enactment. Barriers are known and manageable. Strong agenda synergy with 4+ other ARA proposals.	≥1 Tier A or 2 Tier B independent sources supporting the projected trajectory. CBO or PWBM analysis available or can be requested. Fiscal multiplier ≥1.0 where applicable.	No constitutional barrier. Some administrative complexity manageable within normal government capacity.
3	Moderate Transition Difficulty	Implementation feasible under majority-governance conditions (simple majority vote or executive authority with strong political will). Some US domestic precedent exists, though adaptation is required. Timeline to operational: 5–10 years from enactment. Manageable but real barriers exist. Meaningful agenda synergy with 2–3 other ARA proposals.	≥1 Tier B independent source supporting the trajectory. Economic modeling suggests material positive fiscal or social outcome. Fiscal multiplier ≥0.75 where applicable.	Manageable legislative or administrative barriers. Some coordination complexity.
2	High Transition Difficulty	Implementation faces significant structural barriers: 60-vote Senate threshold, multi-agency coordination requirements, strong and durable organized opposition, or constitutional constraints that limit reach. Limited US domestic precedent at the required scale. Timeline to operational: 10–20 years under optimistic assumptions. Meaningful implementation requires unusually favorable political conditions. Modest agenda synergy.	≥1 Tier C source or domestic program analogy suggesting positive direction. Significant uncertainty acknowledged in scoring rationale.	Significant implementation barriers. Realistic only under unusually favorable political conditions.
1	Extreme Transition Difficulty	Policy faces fundamental constitutional, legal, or political barriers making full implementation unlikely even under optimistic conditions: requires supermajority or constitutional amendment; faces irresolvable administrative capacity constraints; or the US institutional context makes this type of transition structurally infeasible within a 20-year horizon. Score of 1 does not mean the policy is unimportant — it means the transition gap is currently unbridgeable and should not be understated.	Tier C/D evidence or no affirmative modeling. Scoring rationale must explain why trajectory is assessed as minimal despite any positive GP score.	Fundamental barriers. Implementation requires conditions beyond normal democratic governance.

UT Scoring Note: Constitutional Amendment Pathway

Proposals that require a constitutional amendment to achieve their full effect (e.g., DIR Act Track B) should be scored on UT as if the amendment pathway is the operative route — not as if statutory action will achieve the same result. The UT score should reflect the realistic probability-weighted impact: a proposal that needs constitutional amendment but has strong state-level momentum may score UT=2 (limited but achievable long-term), whereas one requiring amendment without any state-level groundwork may score UT=1.

UT Scoring Note: Applying Research Findings to the Correct Axis

The most common error in applying this rubric is attributing evidence about benefit effectiveness to UT rather than B. Example: Prop 17 (National Voting Holiday). The best available research (Farber 2009) finds approximately zero effect on voter turnout from making Election Day a federal holiday. This is a finding about what the policy achieves (B), not about how hard it is to implement (UT). Making Election Day a federal holiday requires simple majority legislation or in some forms an executive proclamation — it is a low-difficulty transition (UT=4). Reducing B to reflect weak outcome evidence is correct. Reducing UT to reflect weak outcome evidence is not — it conflates two orthogonal axes and obscures the real policy insight: a policy that is easy to implement but does not achieve its intended benefit is a different policy problem than one that achieves real benefit but is hard to implement. The UT axis exists to separate these two questions.

Section 5 — Axis 3: Benefit Score (B)

How significant is the welfare benefit to the broad American population if the policy achieves its intended outcomes?

The B axis measures the magnitude and distribution of welfare benefit delivered to Americans if the policy succeeds. It is assessed conditional on success — it is not a probability-weighted expected value (that is captured by UT). B asks: if this works as intended, how good is it?

B is assessed along three dimensions that independently contribute to the score:

- **Population reach:** what percentage of Americans receive a direct, material benefit?
- **Per-beneficiary magnitude:** how substantially does the policy improve the welfare of each beneficiary? This is assessed relative to the baseline without the policy.
- **Distributional equity:** does the policy disproportionately benefit those with the greatest unmet need (progressive), serve the population broadly (universal), or primarily benefit those already well-served (neutral to regressive)?

A policy can achieve a high B score by scoring exceptionally on one dimension and moderately on others — for example, a narrow policy with truly transformative per-beneficiary impact for a specific underserved group may score B=4. The scoring rubric reflects this structure.

Score	Tier Label	Qualifying Criteria (primary condition; see sub-dimensions)	Distributional Standard	Evidence Minimum
5	Maximal Benefit	≥80% of Americans benefit directly with substantial per-beneficiary welfare improvement; OR the policy is foundational infrastructure (healthcare, education, financial security) that enables materially higher quality of life for the large majority of Americans; OR the policy prevents catastrophic harm at national scale (public health, environmental, economic systemic risk).	Universal or strongly progressive. Benefits accessible regardless of income, geography, or demographic status.	Quantified welfare analysis with Tier B/A evidence. Cost per QALY or equivalent metric favorable. Multiple independent analyses converge.
4	Substantial Benefit	60–80% of Americans benefit directly with significant per-beneficiary welfare improvement; OR 30–60% benefit with transformative per-beneficiary impact (e.g., elimination of a major source of financial precarity); AND benefits are meaningfully progressive (concentrated in working and middle-class Americans).	Meaningfully progressive. Benefits disproportionately serve those with greatest unmet need.	Quantified welfare analysis with Tier B evidence. Addresses documented market failure or public goods gap. CBO or equivalent impact estimate available.
3	Meaningful Benefit	30–60% of Americans benefit directly with meaningful per-beneficiary welfare improvement; OR narrower population benefits with transformative per-beneficiary impact; AND distributional pattern is at minimum neutral (does not primarily serve those already well-served).	Progressive or neutral. Does not concentrate benefits among high-income Americans.	Some quantified evidence of welfare improvement. Policy addresses recognized market failure or public goods gap. Tier B/C evidence minimum.

Score	Tier Label	Qualifying Criteria (primary condition; see sub-dimensions)	Distributional Standard	Evidence Minimum
2	Limited Benefit	10–30% of Americans benefit directly with moderate per-beneficiary improvement; OR broader population with modest per-beneficiary impact (incremental improvement rather than structural change). May serve an important function without generating large welfare gains (e.g., procedural or institutional reforms).	Neutral to slightly progressive. No significant regressivity.	Some evidence of welfare improvement (Tier C minimum). Economic analysis suggests moderate positive effect.
1	Narrow Benefit	<10% of Americans benefit directly with modest per-beneficiary welfare improvement; OR policy is primarily institutional/procedural with indirect and diffuse welfare effects; OR policy serves an important enabling function but generates no direct welfare benefit itself. Score 1 does not indicate unimportance — some foundational policies are critical but narrow in direct impact.	Distributional pattern may vary; narrow reach is the primary determinant.	Tier C/D evidence or limited quantified analysis. Scoring rationale must note if the policy's primary value lies in its enabling function rather than direct benefit.

B Scoring Note: Enabling Policies

Several ARA proposals score B=1 or B=2 not because they are unimportant but because their primary value is enabling other proposals to succeed. Address Gerrymandering (Prop 16) and Shutdown Accountability for Congress (Prop 18) are examples: their direct welfare impact is modest, but they protect the democratic infrastructure that makes all other proposals possible. This is the correct score — and should be explained in the scoring rationale, not corrected upward. The aggregate score formula naturally reflects that these enabling proposals combine with high-value delivery proposals to produce a coherent agenda.

Section 6 — Score Calibration Examples

Illustrative applications of the rubric across diverse proposal types

The following examples illustrate how the rubric applies to proposals across the three axes. These are intended as calibration anchors to promote consistency across scorers. Rationale is abbreviated; full per-proposal justifications appear in the companion scoring justification record.

Proposal	GP	UT	B	Score	Abbreviated Rationale
IRS Reform & Pre-Filed Taxes (Prop 1)	4	4	4	8.0	GP=4: 10+ Tier 1 nations use pre-filled returns (OECD Tax Admin 2025; Denmark, Sweden, UK, Australia); Tier A evidence of high satisfaction and compliance (Sources 1,7). UT=4: Revenue Reconciliation vehicle; IRS precedent from \$80B IRA appropriation; 2-3 year to impact; complements BWDS. B=4: Affects all taxpayers; progressive in burden-reduction (lower-income filers benefit most from simplification); reduces tax gap by \$600B/decade.
Socialized Public Childcare (Prop 9)	4	4	5	8.7	GP=4: Quebec, Denmark, Finland, Sweden, Germany — 6+ Tier 1 nations; Tier A evidence from Quebec natural experiment (Sources 42,45,46); fiscal return >1:1 documented. UT=4: Reconciliation-eligible (IRC §21 structure); 3-5 years to meaningful impact; strong synergy with FSW Act and EITC expansion. B=5: Affects 70%+ of American families with children; strong progressive distribution (highest burden relief for lower-income working families); documented labor-force-participation effect.
Address Gerrymandering (Prop 16)	3	2	1	4.0	GP=3: AIRC (Arizona, California), UK BCSE, Canada, Australia — IRCs widely used; Tier B evidence of improved competitiveness; contested whether outcomes improve substantively. UT=2: Requires statutory action (Track A, federal elections) plus post-Callais constitutional concerns for state legislative maps; meaningful implementation within 10 years only in favorable conditions. B=1: Direct welfare impact is narrow and institutional; primary value is enabling democratic accountability and protecting other ARA proposals — a foundational policy.
National Firearm Standards Framework (Prop 21)	2	3	3	5.3	GP=2: Canada, Australia, UK — shall-issue licensing in 2 Tier 1 nations with mixed results; US constitutional constraints (Bruen) limit transferability of international models; genuine US-specific architecture required (FCOC). UT=3: Track B is constitutionally achievable; 5-8 years to meaningful implementation post-enactment; FCOC compliance reaches ~65M acquisition events annually. B=3: Directly affects all gun purchasers (30%+ of population); moderate per-beneficiary impact from certified safety training; progressive in reducing community-level violence burden concentrated in lower-income areas.

Proposal	GP	UT	B	Score	Abbreviated Rationale
Universal Government-Funded Healthcare (Prop 13)	5	2	5	8.0	GP=5: Implemented at national scale in all 16 Tier 1 comparators; OECD endorses universal coverage as a standard; Tier A evidence from multiple longitudinal studies showing superior outcomes at lower cost. UT=2 (High Transition Difficulty): German multi-payer model is the most feasible US path; requires 7+ years of phased implementation; faces ERISA §502 coordination, provider rate-setting challenges, and massive political resistance even under favorable conditions; 60-vote threshold or constitutional obstacles for most architectures. The transition barrier is structural, not merely political. B=5: Benefits all Americans; reduces medical bankruptcy (affecting ~500,000 annually); eliminates coverage gaps; strongly progressive in distributional impact.
Nuclear Diplomacy via Allied Mediation (Prop 65)	3	2	4	6.0	GP=3: Norway, Finland, Switzerland have successful mediation traditions; Tier B evidence from Oslo Accords, Iran nuclear deal; contested in outcome evaluation. UT=2: Requires favorable geopolitical conditions beyond US control; 10+ year timeline for meaningful nuclear risk reduction; dependent on adversarial cooperation that cannot be compelled. B=4: If successful, benefits all Americans through reduced existential risk; the welfare benefit of reduced nuclear risk is by definition maximal in expectation, but the probability-conditional nature of UT (not B) correctly captures the uncertainty.

Section 7 — Limitations and Intellectual Honesty

A scoring methodology that claims more precision than it possesses is less credible, not more. The following limitations apply to this framework and must be understood by anyone using or evaluating the scores.

Inherent Limitations of Cross-National Comparison

The GP rubric controls for institutional comparability using the Tier 1/Tier 2 nation pool, but no control is complete. Even among Tier 1 comparators, the US differs in ways that affect policy outcomes: a federal system of unusually divided and constitutionally constrained government; a healthcare and employment system built around private actors and employer-based benefits; a history of racial inequity that affects policy reach and distribution; and a political economy shaped by campaign finance rules with no close parallel. These differences are acknowledged in the UT axis (which applies US-specific analysis), but they also limit the inferential weight of any single GP score.

Uncertainty in Forward-Looking UT Scores

UT scores require projections about what will happen — a fundamentally uncertain enterprise. Economic models disagree; political conditions change; implementation quality varies enormously. The rubric mitigates this by requiring evidence-based projections and by assigning lower scores where the trajectory is uncertain. But UT scores for proposals with limited modeling (particularly Phase III proposals that are 15+ years from introduction) carry higher uncertainty than scores for Phase I proposals with CBO analysis available. Canonical adoption of this methodology does not eliminate that uncertainty: UT scores for several proposals may require substantive revision as sourcing continues to develop and as the legislative environment evolves.

The Welfare-Benefit Assessment Is Not Value-Neutral

The B rubric makes choices. Distributional progressivity is weighted positively — a policy that primarily benefits the wealthy scores lower on B than one that benefits lower- and middle-income Americans at the same aggregate scale. This reflects the stated mission of the RDNC (reform in the interest of the broad population, not narrow interests) and is a defensible normative choice for a think tank with this mission. But it is a choice. Stakeholders who disagree with the distributional weighting will score B differently. This limitation does not invalidate the rubric; it requires transparency about the normative foundation.

Interrater Reliability

The rubric is designed for reproducibility, but policy scoring retains inherent judgment. Two analysts applying this rubric to the same proposal may arrive at scores that differ by ± 0.5 to ± 1.0 on individual axes, translating to aggregate score differences of up to ± 0.7 . This is acceptable for a think tank framework — it reflects genuine ambiguity in the evidence, not methodological failure. Where adjacent-score ambiguity exists, the scoring rationale should document the ambiguity and explain the resolution. The introduction of this formal rubric is expected to reduce interrater variance compared to the informal methodology used in prior versions.

This Methodology Document Is a Living Document

Scoring methodology in policy research should evolve as the evidence base grows, as implementation experience accumulates, and as the policy agenda develops. This document should be reviewed and updated whenever: (a) the Framework is updated to v27 or later; (b) a major new body of evidence emerges that changes the evidentiary foundation of a proposal's score; (c) the ARA legislative agenda advances and more precise modeling becomes available; or (d) external peer review identifies methodological deficiencies. Each revision should be logged with an audit entry, and prior versions should be retained for reference.

*Reform Democrats of the New Commonwealth · Project ARA 2028 · Developed with the assistance of Claude (Anthropic).
The ideas belong to anyone willing to fight for them.*